

Study supporting the review of the Health Preparedness and Emergency Response Authority (HERA) with regard to its operations, structure, and governance

Executive Summary

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Contact: Lukas LEIBOLD

E-mail: Lukas.LEIBOLD@ec.europa.eu

European Commission
B-1049 Brussels

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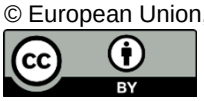
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Executive Summary

The Health Emergency Preparedness and Response Authority (HERA) was established as a Commission service in September 2021 by Commission Decision C(2021) 6712 (European Commission, 2021d) with the objective "*to strengthen Europe's ability to prevent, detect, and rapidly respond to cross-border health emergencies, by ensuring the development, manufacturing, procurement, and distribution of key medical countermeasures*"[1]. As such, HERA's current core mission[2], is to improve the EU's preparedness and response to serious cross-border threats in the area of medical countermeasures ("MCMs").

One of the rationales for establishing the Health Emergency Preparedness and Response Authority was also to provide an EU response to address the challenges experienced by the Member States during the COVID-19 pandemic. More generally, the aim, specifically in the domain of MCMs, was to transform the lessons learnt from the COVID-19 pandemic into structural and systematic change, both in preparedness and response to future health crises. HERA was established to address the following key issues:

1. Fragmentation of efforts in the EU.
2. Weak anticipatory threat and risk assessments, modelling and needs monitoring, suboptimal intervention instruments and public-private ecosystems.
3. Market and supply chain intelligence/market failures in specific contexts.
4. Development, financing, and deployment of new countermeasures in times of crisis.

Article 2(1) of Commission Decision C(2021) 6712 (European Commission, 2021d, p. 2) states that HERA should work to improve preparedness and response by:

- Strengthening health security coordination within the Union during preparedness and crisis response, and bring together the Member States, the industry, and the relevant stakeholders in a common effort.
- Addressing vulnerabilities and strategic dependencies within the Union related to the development, production, procurement, stockpiling and distribution of medical countermeasures.

[1] See link and also link .

[2] As stated in Article 2(1) of Decision 6712.

- Contributing to reinforcing the global health emergency preparedness and response architecture.

Article 8 of the Decision establishing HERA (in addition to several other regulations[3]), sets the legal basis for the review of HERA, to which this study contributes. The objective of this study is to review the operations of HERA, including its structure and governance – forming one component of the wider review process that will be conducted by the Commission. The data and analysis presented in this report is intended to support the Commission to assess:

- The implementation of the operations of HERA, including its structure and governance.
- The potential need to modify the mandate of HERA and the financial implications of any such modification.
- The potential need to establish HERA as a distinct entity, considering relevant agencies or authorities active in the field of health preparedness and response, including those active in the field of health crisis response.

Methodological Approach

The 6-month study delivered this analysis through the execution of the following distinct tasks:

- **Task 0 - Inception phase:** aimed to advance and refine the methodological approach to the study through preliminary desk research and four exploratory interviews.
- **Task 1 - Desk Research**: aimed to 'identify and collect existing qualitative and quantitative evidence and propose the methodological framework to assess it'. This was divided into three subtasks:
    - Sub-task 1.1 focused on HERA's activities and operations, communication, policy documents, and other sources that the Commission made available.
    - Sub-task 1.2 reviewed the policy and legal framework.
    - Subtask 1.3 focused on the assessment framework. The sources gathered were analysed to formulate preliminary answers to the study questions.

[3] Namely: Article 33 of Regulation (EU) 2022/2371 on Serious Cross-Border Threats to Health and Article 16 of Council Regulation (EU) 2022/2372 on a framework of measures for ensuring the supply of crisis-relevant medical countermeasures in the event of a public health emergency at Union level.

- **Task 2 – Consultation Activities**: launched in November 2023 and closed on 19 February 2024. Four separate consultation activities were conducted to collect data, including:
    - Sub-task 2.1a Public Consultation[4].
    - Sub-task 2.1b Call for Evidence[5].
    - Sub-task 2.2 Targeted Surveys[6].
    - Sub-task 2.3 Targeted Interviews[7].
- **Task 3 – Analysis**: analysed all information collected in the above tasks, comprising the final report of this study.
    - Sub-task 3.1 focused upon the achievements and performance of HERA's operations.
    - Sub-task 3.2 on the legal framework.
    - Sub-task 3.3 on the governance framework.

The key questions addressed in the study are summarised in the table below.

[4] 44 replies and 18 attached documents were received.

[5] 47 replies and 15 attached documents were received.

[6] A total of three online surveys have been launched targeting three different categories of stakeholders (685 in total). The surveys received a total of 210 responses. After the cleaning of the dataset (removal of duplicate responses and incomplete answers) 117 replies were analysed.

[7] A total of nine in depth interviews were conducted with six Member of the European Parliament (MEPs), two representatives of EU agencies, and one special advisor to the Commission.

Table 1 – Key study questions

Domain	Evaluation criteria	Overarching study question
Overall performance assessment	**Relevance**	Has the work performed so far by HERA confirmed the need to strengthen health security coordination within the Union in the area of medical countermeasures in times of crisis and preparedness ahead of crises?
	Effectiveness	Has HERA delivered effectively on its mission and tasks in general and were HERA's operations successful in progressing towards its objective to strengthen the EU's health emergency preparedness and response in the field of medical countermeasures (MCMs)?[3]
	Efficiency	Has HERA delivered efficiently on its mission and tasks?
	Coherence	How well (or not) has HERA interacted with other interventions at different levels (internal, external, international)?
	EU added value	Has HERA demonstrated EU added value in performing its activities?
Domain	**Dimensions**	**Overarching evaluation question**
Mission and mandate	**Legal framework**	To what extent has the existing legal framework currently in place enabled HERA to pursue its mandate effectively and efficiently? Can the current legal framework and HERA's mandate be improved, making HERA more 'fit for purpose'?
Governance	**Governance and organisational set-up**	To what extent has the current organisational set-up and governance structure enabled HERA to act, make decisions, and engage with relevant public and private stakeholders timely, efficiently, and effectively? Can HERA's current organisational set up and governance structure be improved, making HERA more 'fit for purpose'?

It should be noted that this study represents just one component of the wider review process that will be carried out by the Commission. Although within the scope of the overall review of HERA, the detailed analysis of the complementarity between HERA's work and work carried out by other Commission Services (such as DG SANTE, DG RTD, DG ECHO and DG GROW) is outside the scope of this study. Moreover, in the performance of activities HERA cooperates with and relies often on instruments and resources of these other EU actors. This study consequently can only provide partial insights into the review questions of performance, mandate and governance.

Study findings

Within the limited scope of this study and limitations outlined below by triangulating factual information, secondary sources, and the results of the consultation activities, this study draws the following findings:

Relevance

- The scope and work undertaken by HERA within the boundaries of its mission and tasks as laid down in Commission Decision C(2021) 6712 are relevant at present and are anticipated by stakeholders to continue to be relevant in the future. Recent developments such as population growth, climate change and CBRN[8] threats

[8] Chemical, Biological, Radiological and Nuclear Threats.

highlight the need for continued EU-level action to ensure the EU is prepared and equipped to respond to serious cross-border threats to health (European Commission, 2021b). Recent issues such as the mpox epidemic, the COVID-19 pandemic, and medicine shortages further demonstrate the need for EU-level action.

- The dual mode of HERA, encompassing both health emergency preparedness and response, is considered highly relevant. HERA's ability to shift into crisis mode in the event of an emergency is considered necessary to effectively utilise the actions taken in the preparedness phase.
- HERA's objectives were pursued in ways that were aligned with wider EU policy goals in the field of health security.
- Recent challenges that gained political momentum, such as the shortages of medicines, are not formally within HERA's original mandate as non-crisis medicines are not considered medical countermeasures required to respond to serious cross-border threats to health.

Effectiveness[9]

- HERA, in its current organisational form and while collaborating with other Commission services and bodies, is progressing towards achieving its objectives.
- The main outputs produced by HERA to date address the three specific objectives as defined in HERA's mission (Article 2(1) of Commission Decision C(2021) 6712) and are in line with HERA's work plans. The outputs of HERA that have not yet been delivered appear to be subject to only a limited delay.
- HERA has contributed to improved horizon scanning capacity at the EU level.
- HERA supported the advancement of late-stage research and development of MCMs. However, stakeholders believe that HERA could be more effective by having more flexible sources of funding.
- HERA's actions have resulted in improved production and manufacturing capacities for MCMs in the EU.
- HERA has contributed to more effective procurement of MCMs and improved MCM stockpiling and distribution capacities at EU level, when compared to actions taken by Member States acting alone.
- HERA has launched several activities on the international stage.

[9] Note that, since HERA is part of the Commission, it does not act in isolation and its results have been achieved in cooperation with other services across the Commission, particularly SANTE, RTD, ECHO as well as with EU agencies such as EMA and ECDC.

- HERA has contributed to the improvement of EU health security coordination by collaborating with European Medicine Agency (EMA), European Centre for Disease Prevention and Control (ECDC) and the Member States. In combination, these actors identified three high-impact health threats.[10] HERA also collaborated with EMA to establish the Critical Medicines Alliance. Based on these activities, HERA is contributing to alignment and close collaboration on EU and global health security.
- HERA results cannot be achieved without a strong cooperation with other Commission services and agencies, particularly with DGs SANTE, ECHO, RTD, and with ECDC and EMA. In its work on critical medicines, HERA cooperates closely with DG SANTE and DG GROW, as well as with EMA.

Efficiency

- HERA, as part of the Commission and while collaborating with other services and relying on HaDEA to implement its activities under the EU4Health programme and the Health Cluster of Horizon Europe, has fulfilled most of the actions detailed in its work plans within the initially foreseen budget.
- HERA's existing funding mechanisms are perceived by stakeholders to present inefficiencies due to the fact that HERA does not manage its funding directly. However, HERA has a very important role in the design of the work programmes and of different calls, but this can vary between funding programmes.

Coherence

- HERA has performed its tasks coherently with actions carried out by other EU actors, however, more can be done to address perceived overlaps, particularly with EMA and ECDC. Stakeholders consider that more can be done to clarify the responsibilities and to avoid duplication of efforts.
- HERA has made efforts to coordinate with the Member States' activities at national level through the HERA Board and other consultation mechanisms.

EU added value

HERA demonstrated EU added value, also the result of collaboration with other Commission services, by contributing to:

- Enhanced EU preparedness and response capabilities, through cost-effective procurement of MCMs that ensured equitable access across Member States.

[10] Pathogens with high pandemic potential; chemical, biological, radiological, and nuclear threats; and antimicrobial resistance.

- A more coordinated approach to the funding of R&D of MCMs, through the prioritised and encompassing focus of the published calls.
- The creation of financing capabilities to address market challenges based on pooled resources.
- The coordination of stockpiles, including establishment of strategic EU stockpiles under rescEU as a last resort measure.
- Quick response to the issue of critical medicine shortages showcasing EU added value compared to individual actions that Member States may have taken.
- Collaboration with global actors, including supporting WHO initiatives in a way that Member States could not perform individually, thus contributing toward a robust health security framework for the EU.

Legal and Governance frameworks

In the analysis of the **legal framework and mandate**, some gaps and overlaps emerge regarding the adequacy of HERA to act during crisis and/or preparedness effectively and efficiently. These include:

- There is a perceived blurred demarcation between the mandates of ECDC / EMA and HERA when it comes to data and information gathering for what concerns assessment of health threats and supply chain intelligence, which may reduce HERA's efficiency.
- A potential gap is the limitation of HERA's mandate to cross-border health threats in the area of MCMs, in EU legal acts[11], which is particularly relevant for the domain of critical medicines.

In addition, the difficulty to distinguish between more general health preparedness actions and emergency health preparedness one in MCMs has been pointed out.

The upcoming review of HERA to be conducted by the Commission offers the opportunity to further examine these issues and explore potential amendments and modifications that could address the identified gaps and overlaps, taking into account the broader legal context in which HERA operates.

HERA's current **governance structure**, both as defined in Articles 3 to 7 of Commission Decision C(2021) 6712 and as a result of HERA being a Commission service as defined in Article 1 of the Decision, has enabled an effective coordination with Member States, other Commission services, decentralised EU agencies, and stakeholders' groups. While certain

[11] Namely, Regulation (EU) 2022/2371 on Serious Cross-Border Threats to Health and Council Regulation (EU) 2022/2372 on a framework of measures for ensuring the supply of crisis-relevant medical countermeasures in the event of a public health emergency at Union level.

stakeholders believe that not all relevant stakeholders are currently included in HERA's governance structure, HERA has put together several mechanisms and communication channels to ensure all relevant stakeholders are involved. With respect to transparency and accountability, as a Commission service, HERA complies with measures applicable to all Commission services. Despite this, some of the consulted stakeholders still consider that HERA does not sufficiently communicate about its activities.

With regard to its funding mechanisms, some of the consulted stakeholders and also analysts (i.e., Renda et al 2023a; Renda et al 2024) expressed the opinion that HERA could be more effective and efficient especially in time of crisis if its budget was more unified (less fragmented) and flexible[12]. Whilst stakeholders believe that the current governance structure and funding mechanisms have enabled HERA to pursue its mandate efficiently and effectively, it is thought that this could be increased with a dedicated budget line for HERA activities providing more financial flexibility, as well as adequate funding and legal basis for using such funds in line with the need to address preparedness and response. However, the policy trade-off of creating a dedicated budget line is the risk of budgetary fragmentation and potential silo creation which might, instead, be limiting the flexibility to react to different priorities. Moreover, any reflection about the future budget for HERA needs to take into account the overall set-up of the future MFF.

Finally, based on the analysis and the available evidence, the study presents the analysis of Pros and Cons with regard to two issues: the extension of HERA's mandate and its organisational form (Commission services as opposed to other EU bodies and agencies.

The decision to modify HERA's mandate extending it as to fully include in it the issue of shortages of critical medicines, as well as the decisions to address other identified gaps, or the refinement of the mandate to address overlaps and ambiguities in other areas, entails considering the trade-off between what can be considered as improvement and the debate and disruption needed to implement them. While it seems that the improvements may be worth the efforts, in line with the principle of 'One Commission', it is up to the Commission's internal decision how to distribute the tasks of its services. A decision depending on the Commission's overall policy objectives and internal governance considerations.

The decision to establish HERA as a service within the European Commission versus an independent EU body or agency involves weighing the trade-offs between integration and autonomy, political oversight and independence, agility and predictability, coordination fragmentation, as well as innovation and specialisation. The different options offer distinct advantages and challenges, and the choice will depend on various factors, including the EU's overall policy objectives, governance considerations, potential overlaps and synergies

[12] In their free text replies stakeholders stated that more budgetary autonomy/flexibility is needed, without explaining what autonomy/flexibility means. Our interpretation is that by autonomy is meant that HERA has a dedicated funding mechanism (instead of relying on other existing sources) and not autonomy from the Commission. Flexibility, on the other hand, can be interpreted as meaning that HERA needs to have access under the next MFF to funding that fits its needs also in times of crisis. See more considerations on this aspect in Section 2.3.

(or lack thereof) with existing EU bodies or decentralised and executive agencies and the specific needs of health emergency response and preparedness efforts. As highlighted by some of the consulted stakeholders, irrespective of whether HERA is a Directorate-General within the European Commission or an independent EU body or agency, it should avoid overlapping mandates with existing EU actors, ensure an effective and continuous cooperation and ensure it fulfils its mission and mandate efficiently and effectively.

Limitations of this study

The overall findings of the study presented in this report are constrained by a lack of retrievable quantitative evidence to date on HERA's outputs and impacts. The only factual information used in this study to assess performance are HERA workplans and activity reports. The lack of evidence on outputs and impacts can be attributed to HERA only becoming operational in 2022. As a result, the delivery of some of HERA's outputs is still ongoing, meaning an analysis of their impact is outside the timing of this study. Therefore, the assessment of the criteria of effectiveness focuses only on progress toward objectives without considering effect and impacts, whereas that of efficiency and EU added value is based on the triangulation of the limited available factual information and the findings of the consultation activities. Two additional disclaimers concern the consultation activities. First, the consultation focussed mostly on external stakeholders and, so, the study captures external rather than internal views on HERA. Second, while efforts were made to reach as many stakeholders as possible, stakeholders that regularly interact with HERA responded more to the Public Consultation and to the targeted surveys compared to other relevant groups such as especially stakeholders from the research community that have been only limitedly reached in the targeted survey. The limited responses from the research community stakeholders must be taken in considerations when reading the descriptive statistics from the two consultations.

Next, it must be stressed that the policy area of critical medicine has been developing as the study was progressed and could only be partially reflected in it.

Finally, it should be noted that as part of the Commission, HERA collaborates both with other Commission services where relevant, primarily with DGs SANTE, ECHO, RTD and GROW, and Union agencies ECDC and EMA. Thus, outputs may often be considered as a result of joint collaboration. However, based upon the scope of the study and the limited information retrieved it was beyond this study's scope to systematically identify and attribute the different contributions.

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